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Data Review

Data Citation

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Abstract

The International Reproduction Policy Database (IRPD) is a novel dataset that captures national-level policies directly regulating human reproduction across 33 high- and middle-income countries between 1980 and 2020. Developed by the Emmy Noether Research Group “Varieties of Reproduction Regimes” at the WZB Berlin Social Science Center, the IRPD systematically covers five areas: sex education, contraception, abortion, assisted reproductive technologies, and pregnancy care. Data were collected through expert questionnaires, ensuring both breadth and depth of policy coverage. The database offers conceptual and methodological innovations, including its exclusive focus on statutory policies and its transparent documentation practices. However, some dataset limitations, such as restricted sub-national data, challenges of external validation, and occasional gaps in coverage, should be acknowledged. While only Estonia, Latvia, and Lithuania are currently represented among Discuss Data-relevant countries the IRPD offers significant potential for comparative and longitudinal research on reproductive governance, policy interrelations, and institutional effects on social norms.

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Data Review – International Reproduction Policy Database (IRPD)

The International Reproduction Policy Database (IRPD) is a comprehensive dataset that systematically measures policies directly regulating human reproduction across high- and middle-income countries (3). Developed as “a subproject of the research group ‘Varieties of Reproduction Regimes’ at the WZB Berlin Social Science Center” (3), the IRPD data covers – as of September 2025 – the period from 1980 to 2020 and includes 33 countries (3). The database was firstly available in February 2025 (3) and provides national-level indicators in five regulatory domains – “sex education, contraception, abortion, assisted reproductive technologies, and pregnancy care” (3). The data were collected through expert questionnaires that were pretested with field specialists to ensure the inclusion of “the most relevant aspects of regulation in all fields” (7). By offering a standardised, longitudinal, and cross-national perspective on reproduction policies, the IRPD constitutes a novel tool for analysing state regulation, inequalities, and social norms around reproduction (1; 3).

Methodological Foundation

The methodological foundation of the IRPD is based on systematic data collection through expert questionnaires covering five key policy areas (5; 6). The questionnaires were first pretested with field experts, who provided feedback to refine the items and ensure “the most relevant aspects of regulation in all fields” (7) were included across all domains (7). Data collection was then carried out by “country experts in 33 high-income countries and one historical country case, the German Democratic Republic (1980-1990)” (8), who reported on national-level regulations for every year between 1980 and 2020 (5; 8). This process ensured both the breadth of coverage across countries and decades, as well as the depth and accuracy of the policy indicators included in the data set.

Critique

Zagel et al. (2025) present the IRPD as a novel and conceptually grounded tool for the comparative study of reproduction policies (Zagel et al. 2025c: 210). The authors – themselves involved in the creation of the IRPD – provide both a strong rationale for its necessity and an honest view on its limitations.

A key strength of the IRPD lies in its conceptual innovation. Unlike previous approaches that often included reproduction policies under broader categories such as population or morality policy, or that blended policies with outcomes (Zagel et al. 2025c: 212), the authors of the IRPD “propose a focus on the institutional level of statutory policies [...] focus[sing] on policies by which states directly intervene in the biological and social processes simultaneously involved in reproduction up until birth” (Zagel et al. 2025c: 214).

By distinguishing five interrelated policy fields, the database allows for systematic analysis of how states regulate reproduction across different life course stages (Zagel et al. 2025c: 215). The differentiation into “regulatory levels, regulatory types, permissiveness, in-kind generosity and in-cash generosity” (Zagel et al. 2025c: 214) is especially relevant for identifying variations both within and across countries (Zagel et al. 2025c: 212).

Another contribution is methodological thoroughness in data collection. The authors recruited national experts in 33 countries, required them “to rely on policy documents such as legal texts, official guidelines or court decisions” (Zagel et al. 2025c: 216), and ensured transparency by storing all source documents “with the dataset for potential follow-up investigations” (Zagel et al. 2025c: 216). This procedure strengthens the replicability and credibility of the data (Zagel et al. 2025c: 216). Moreover, by spanning four decades (1980-2020), the IRPD enables the study of both cross-national patterns and longitudinal changes, thus filling a major empirical gap in welfare state and reproductive policy research (Zagel et al. 2024: 216).

Nevertheless, the database also has limitations, which the authors openly acknowledge. First, validity and reliability issues remain, as the limited resources for data collection prevented detailed verification of all country inputs (Zagel et al. 2025c: 216-217). Second, the focus on national-level regulation excludes sub-national variations, which may be substantial in federal states or decentralised systems (Zagel et al. 2025c: 215-216). This centralisation improves comparability but risks overlooking important within-country disparities. Third, the lack of other comprehensive datasets on reproduction policy makes external validation difficult, limiting the ability to fully assess measurement accuracy (Zagel et al. 2025c: 215-217).

Despite these challenges, the IRPD provides important opportunities for future research. Its comprehensive structure allows for the exploration of policy interrelations – such as complementarities or substitution effects between fields – and for testing hypotheses about convergence, divergence, or the role of political institutions (Zagel et al. 2024: 218).

Using the IRPD

As of September 2025, access to the IRPD dataset is provided through the GESIS data archive. Researchers are required to register by providing their first and last name, country of residence, area of expertise, e-mail address, and a password in order to get access.

The dataset is available for download in .dta format. In addition, a comprehensive codebook in PDF format can also be accessed via GESIS to facilitate understanding and use of the data.

Applicability for Countries Relevant to Discuss Data

Among the 15, for Discuss Data relevant countries, as of September 2025 only four are covered by the IRPD: Estonia, Latvia, Lithuania, and Ukraine. However, for Ukraine no data is currently available (Zagel et al. 2025a; Zagel et al. 2025b), which limits its immediate usability for comparative research in this context.

In order to check for the data coverage, an exemplary review of the abortion domain was carried out (see table 1). For the three Baltic states, the database provides systematic information in this category, including legal grounds for termination, procedural requirements such as spousal or judicial consent, and institutional settings for the provision of abortion services. In Latvia, some indicators cover the entire research period, while others are only available from the early 1990s onward; the same applies to Estonia. Lithuania, by contrast, has more limited coverage, with over half of the checked indicators marked as not available (N/A). Overall, the completeness of information varies: some indicators are fully available, others only cover selected years, and some are marked as N/A. Ukraine, although listed as a country in the codebook, contains no recorded entries in the dataset. (Zagel et al. 2025a)

Table 1 illustrates these dynamics using abortion regulation as an example. While it does not represent the full scope of the dataset, it tries to reflect the general structure of available codes and highlights patterns of partial and missing coverage.

In sum, the IRPD allows for the exploration of longitudinal shifts and reproductive governance in Estonia, Lithuania, and Latvia and facilitates cross-country comparisons of regulatory paths. At the same time, the absence of data for Ukraine and the lack of coverage for other Discuss Data-relevant states such as Armenia, Georgia or Russia underline both the value of the dataset for analysing selected cases and its limitations in capturing the broader post-Soviet or Eastern European region. (Zagel et al. 2025a)

Table 1: Discuss Data Relevant Country Coverage – an example of one of the five topic areas: Abortion (as of September 2025)

Country	Codes with Data for the years 1980-2020	Codes without Data (N/A) but listed in the dataset
Estonia (EE)	<i>AB1_F</i> Between 1980 and 2020, was induced abortion regulated on the national level in any year in your country? (Zagel et al. 2025b: 7)	<i>AB2a</i> Between 1980 and 2020, please indicate the pregnancy weeks up to which induced abortion was allowed in case of saving a woman's life (Zagel et al. 2025b: 11-12)
	<i>AB1</i> Please indicate the years in which abortion was regulated on the national level. (p. 8)	<i>AB2c</i> Between 1980 and 2020, please indicate the pregnancy weeks up to which induced abortion was allowed

		in case of preserving a woman's mental health. (p. 12-13)
<i>AB2_F</i> In the years in which abortion was regulated on the national level, was abortion legal in any year between 1980 and 2020 in your country? (p. 8)	<i>AB2d</i> Between 1980 and 2020, please indicate the pregnancy weeks up to which induced abortion was allowed in case of rape or incest. (p. 13)	
<i>AB2a_F</i> Please indicate the correct answer for your country: In any year between 1980 and 2020, the state allowed access to induced abortion in case of ... saving woman's life. (p. 8-9)	<i>AB2f</i> Between 1980 and 2020, please indicate the pregnancy weeks up to which induced abortion was allowed in case of social or economic reasons (p. 14)	
<i>AB2b_F</i> (see AB2a_F) ... woman's physical health (p. 9)	<i>AB2a1</i> Between 1980 and 2020, please indicate the pregnancy weeks up to which induced abortion was allowed in case of saving a woman's life (p. 15)	
<i>AB2c_F</i> (see AB2a_F) ... woman's mental health (p. 9)	<i>AB2c1</i> Between 1980 and 2020, please indicate the pregnancy weeks up to which induced abortion was allowed in case of preserving a woman's mental health. (p. 16)	
<i>AB2d_F</i> (see AB2a_F) ... rape or incest (p. 10)	<i>AB2d1</i> Between 1980 and 2020, please indicate the pregnancy weeks up to which induced abortion was allowed in case of rape or incest. (p. 16)	
<i>AB2e_F</i> (see AB2a_F) ... fetal impairment (p. 10)	<i>AB2f1</i> Between 1980 and 2020, please indicate the pregnancy weeks up to which induced abortion was allowed in case of social or economic reasons (p. 17)	
<i>AB2f_F</i> (see AB2a_F) ... social or economic reasons (p. 11)	<i>AB3c</i> Please indicate the years in which spousal consent for adults was required to access an abortion. (p. 22)	
<i>AB2g_F</i> (see AB2a_F) ... on request (p. 11)	<i>AB3d</i>	

		Please indicate the years in which legal proof of rape or incest was required to access an abortion. (p. 22-23)
<i>AB2b</i> Between 1980 and 2020, please indicate the pregnancy weeks up to which induced abortion was allowed in case of preserving a woman's physical health. (p. 12)	<i>AB3e</i> Please indicate the years in which ultrasound prior to an abortion was required to access an abortion. (p. 23)	
<i>AB2e</i> Between 1980 and 2020, please indicate the pregnancy weeks up to which induced abortion was allowed in case of fetal impairment. (p. 13-14)	<i>AB3g</i> Please indicate the years in which a specific waiting period between counseling and abortion was required to access an abortion. (p. 24)	
<i>AB2g</i> Between 1980 and 2020, please indicate the pregnancy weeks up to which induced abortion was allowed in case of on request. (p. 14-15)	<i>AB3h</i> Please indicate the years in which additional medical tests, e.g. HIV/STIs were required to access an abortion. (p. 24)	
<i>AB2b1</i> Between 1980 and 2020, please indicate the pregnancy weeks up to which induced abortion was allowed in case of preserving a woman's physical health. (p. 15)	<i>AB4d</i> Please indicate the years in which it was legal for {AB3c_other} to perform abortions. (p. 28)	
<i>AB2e1</i> Between 1980 and 2020, please indicate the pregnancy weeks up to which induced abortion was allowed in case of fetal impairment. (p. 16-17)	<i>AB6a</i> Please indicate the years in which there was full public health insurance coverage of costs for induced abortion between 1980 and 2020 in your country. (p. 33-34)	
<i>AB2g1</i> <i>Between 1980 and 2020, please indicate the pregnancy weeks up to which induced abortion was allowed in case of on request. (p. 17)</i>	<i>AB7a_F</i> If costs were fully covered, was the full coverage of costs dependent on the pregnancy week? (p. 34-35)	
<i>AB3a_F</i> Between 1980 and 2020, which one of the following administrative or medical requirements to accessing abortion services were in place in any year? ... consent from more than one doctor (p. 18)	<i>AB7a</i> Between 1980 and 2020, please indicate the pregnancy week up to which induced abortion was fully covered. (p.35)	

<i>AB3b_F</i> (see AB3a_F) ... judicial or parental consent for minors (p. 18)	<i>AB7b</i> Between 1980 and 2020, please indicate the pregnancy week up to which induced abortion was partially covered. (p. 36)
<i>AB3c_F</i> (see AB3a_F) ... spousal consent (p. 18-19)	<i>AB9c</i> Please indicate the years in which a prescription was necessary to obtain access to medical abortion. (p. 40-41)
<i>AB3d_F</i> (see AB3a_F) ... legal proof or rape or incest (p. 19)	<i>AB9c1_F</i> Between 1980 and 2020, how was the access to medical abortion made available? Please tick all answers that apply. (p. 41)
<i>AB3e_F</i> (see AB3a_F) ... ultrasound (p. 19-20)	<i>AB9c2_F</i> Between 1980 and 2020, how was the access to medical abortion made available? Please tick all answers that apply ... prescription through healthcare professional (p. 41)
<i>AB3f_F</i> (see AB3a_F) ... counseling (p. 20)	<i>AB9c3_F</i> (see AB9c2_F) ... prescription through telemedicine (p. 42)
<i>AB3g_F</i> (see AB3a_F) ... waiting period (p. 20)	<i>AB9c1</i> Please indicate the years in which medical abortion was available through the prescription by a doctor. (p. 42-43)
<i>AB3h_F</i> (see AB3a_F) ... additional medical tests (p. 21)	<i>AB9c2</i> Please indicate the years in which medical abortion was available through prescription by a healthcare professional. (p. 43)
<i>AB3a</i> Data available for the years 1998-2020: Please indicate the years in which consent from more than one doctor was required to access an abortion. (p. 21)	<i>AB9c3</i> Please indicate the years in which medical abortion was available through prescription via telemedicine. (p. 43-44)
<i>AB3b</i> Please indicate the years in which judicial or parental consent for	<i>AB9c4</i> Please indicate the years in which medical abortion was available through {open entry}. (p. 44)

	minors was required to access an abortion. (p. 21-22)	
	<i>AB3f</i> Data available for the years 1998-2020: Please indicate the years in which counseling was required to access an abortion. (p. 23-24)	<i>AB10a</i> Please indicate in which years it was fully restricted to provide information about abortion. (p. 46-47)
	<i>AB4a_F</i> Between 1980-2020, in which facilities was it legal to perform abortions in any year? ... hospitals (p. 25)	<i>AB10b</i> Please indicate in which years there were legal restrictions on providing full information about abortion (partial restriction, as indicated before). (p. 47)
	<i>AB4b_F</i> (see AB4a_F) ... community-based clinics (p. 25)	
	<i>AB4c_F</i> (see AB4a_F) ... doctors' practices (p. 25-26)	
	<i>AB4d_F</i> (see AB4a_F) ... other (e.g. private clinics, women's consultation clinics...) (p. 26)	
	<i>AB4a</i> Please indicate the years in which it was legal to perform abortions in hospitals. (p. 26)	
	<i>AB4b</i> Please indicate the years in which it was legal to perform abortions in community-based clinics. (p. 27)	
	<i>AB4c</i> Please indicate the years in which it was legal to perform abortions in doctor's practices. (p. 27)	
	<i>AB5_F</i> Between 1980 and 2020, did doctors have the possibility to object to carry out an abortion in any year (conscientious objection)? (p. 28)	
	<i>AB5</i> Data available for the years 1998-2020: Please indicate the years in which doctors had the possibility to object	

	to carry out an abortion in your country. (p. 28-29)	
	<i>AB5a</i> Please indicate the years in which doctors were legally obliged to state a reason for objecting to perform an abortion in your country (p. 29)	
	<i>AB5a1_F</i> Please mark the reasons for which doctors could object an abortion in some or all the years in which doctors could object an abortion in your country ... medical concerns (p. 29-30)	
	<i>AB5a2_F</i> (see <i>AB5a1_F</i>) ... ethnical concerns (p. 30)	
	<i>AB5a3_F</i> (see <i>AB5a1_F</i>) ... other (p. 30)	
	<i>AB5a1</i> Please indicate the years in which doctors could object an abortion due to ethical concerns. (p. 31)	
	<i>AB5a2</i> Please indicate the years in which doctors could object an abortion due to medical concerns (p. 31)	
	<i>AB5a3</i> Please indicate the years in which doctors could object an abortion due to other reasons. (p. 32)	
	<i>AB6a_F</i> Between 1980 and 2020, was induced abortion either fully and/or partially covered by public health insurance in any year? ... fully covered (p. 32)	
	<i>AB6b_F</i> (see <i>AB6a_F</i>) ... partially covered (p. 32-33)	
	<i>AB6c_F</i> (see <i>AB6a_F</i>) ... not covered (p. 33)	
	<i>AB6b</i>	

	Data available for the years 1995-2020: Please indicate the years in which there was partial public health insurance coverage of costs for induced abortion between 1980 and 2020 in your country. (p. 34)	
	<i>AB7b_F</i> If costs were partially covered, was the partial coverage of costs dependent on the pregnancy week? (p. 35)	
	<i>AB8_F</i> Between 1980 and 2020, were post-abortion care services covered by public health insurance in your country? (p. 36)	
	<i>AB8a</i> Please indicate the years in which there was full public health insurance coverage of costs for post-abortion care services between 1980 and 2020 in your country. (p. 36-37)	
	<i>AB8b</i> Data available for the years 1991-2020: Please indicate in which years there was partial public health insurance coverage of costs for post-abortion care services between 1980 and 2020 in your country. (p. 37)	
	<i>AB9_F</i> Between 1980 and 2020, was medical abortion at home (use of ‘abortion pill’) legal in your country in any year? (p. 37-38)	
	<i>AB9</i> Data available for the years 2005-2020: Please indicate in which years medical abortion at home was legal in your country. (p. 38)	
	<i>AB9a_F</i> In the years when it was legal, was there a restriction on how long	

	into pregnancy medical abortion at home may be used? (p. 38)	
	<i>AB9a</i> Data available for the years 2005-2020: Please indicate in which years there was a restriction on how long into pregnancy medical abortion may be used (p. 38-39)	
	<i>AB9b_F</i> Between 1980 and 2020, was medical abortion covered by national public health insurance in your country? (p. 39)	
	<i>AB9b</i> Data available for the years 2005-2020: Please indicate in which years medical abortion was covered by national public health insurance. (p. 39-40)	
	<i>AB9c_F</i> Between 1980 and 2020, was the access to medical abortion dependent on a prescription (by a doctor or any other healthcare professional) in any year in your country? (p. 40)	
	<i>AB9c4_F</i> (see AB9c2_F) ... prescription through other (p. 42)	
	<i>AB10_F</i> Between 1980 and 2020, was there a national law or legal guidelines requiring the (national) government/public authorities to provide medically accurate information on abortion and abortion services? (p. 44-45)	
	<i>AB10</i> Data available for the years 1996-2020: Please indicate the years in which the (national) government/public authorities were required to provide medically accurate information about abortion and abortion services. (p. 45)	

	<i>AB10a_F</i> Between 1980 and 2020 was providing information on abortion restricted in your country in any year? ... full restriction (p. 45)	
	<i>AB10b_F</i> (see AB10a_F) ... partial restriction (p. 46)	
	<i>AB10c_F</i> (see AB10a_F) ... no restriction (p. 46)	
	<i>AB10c</i> Please indicate in which years it was legal to provide information about abortion (e.g. by doctors, organisations...). (p. 47)	
	<i>AB11_F</i> Do the government and public authorities take action against abortion disinformation? (p. 48)	
Latvia (LV)	AB1_F AB1 AB2_F AB2a_F AB2b_F AB2c_F AB2d_F AB2e_F AB2f_F AB2g_F AB2g AB2g1 AB3a_F AB3b_F AB3c_F AB3d_F AB3e_F AB3f_F AB3g_F AB3h_F AB3a AB3b AB3d AB3g AB3h	AB2f AB2f1 AB3c AB3e AB3f AB4b AB4c AB4d AB5a3_F AB5a1 AB5a2 AB5a3 AB6b AB7b_F AB7a AB7b AB9b_F AB9b AB10a AB10b

	AB4a_F AB4b_F AB4c_F AB4d_F AB4a AB5_F AB6a_F AB6b_F AB6c_F AB7a_F AB8_F AB8a AB8b AB9_F AB9c_F AB9c1_F AB9c2_F AB9c3_F AB9c4_F AB10_F AB10 AB10a_F AB10b_F AB10c_F AB10c AB11_F	
	Data only available for the years 1991-2020 AB2a AB2b AB2c AB2d AB2e AB2a1 AB2b1 AB2c1 AB2d1 AB2e1 AB9a_F Data only available for the years 1997-2020 AB5 AB5a Data only available for the years 1998-2020	

	AB6a Data only available for the years 2008-2020 AB9 AB9a AB9c AB9c1 AB9c2 AB9c3 AB9c4 Data only available for the years 1980-1996 AB5a1_F AB5a2_F	
Lithuania (LT)	AB1_F AB1 AB2_F AB2a_F AB2b_F AB2c_F AB2d_F AB2e_F AB2f_F AB2g_F AB2g AB2g1 AB3a_F AB3b_F AB3c_F AB3d_F AB3e_F AB3f_F AB3g_F AB3h_F AB3b AB4a_F AB4b_F AB4c_F AB4d_F AB4a AB4d AB5_F AB5a AB6a_F AB6b_F	AB2a AB2b AB2c AB2d AB2e AB2f AB2a1 AB2b1 AB2c1 AB2d1 AB2e1 AB2f1 AB3a AB3c AB3d AB3e AB3f AB3g AB3h AB4b AB4c AB5a1_F AB5a2_F AB5a3_F AB5a1 AB5a2 AB5a3 AB6b AB7b_F AB7a AB7b

	AB6c_F AB7a_F AB8_F AB8a AB8b AB9_F AB10_F AB10a_F AB10b_F AB10c_F AB10c AB11_F Data only available for the years 1997-1996 AB5 Data only available for the years 1994-1996 AB6a	AB9 AB9a_F AB9a AB9b_F AB9b AB9c_F AB9c AB9c1_F AB9c2_F AB9c3_F AB9c4_F AB9c1 AB9c2 AB9c3 AB9c4 AB10 AB10a AB10b
Ukraine (UA)	No data available	
Relevant countries for Discuss Data that are not mentioned in the dataset: Armenia (AM), Azerbaijan (AZ), Belarus (BY), Georgia (GE), Kazakhstan (KZ), Kyrgyzstan (KG), Moldova (MD), Russia (RU), Tajikistan (TJ), Turkmenistan (TM), Uzbekistan (UZ)		

(sources: Zagel et al. 2025a; Zagel et al. 2025b)

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Note

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